

Cardiology Service Line Handbook

Meridian Health Partners - Cardiology Service Line Handbook

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1.0 Purpose

1.1 This handbook defines the scope of services, operational parameters, and patient engagement protocols for the Cardiology department at Meridian Health Partners.

1.2 It serves as a directive for all clinical and administrative staff, ensuring consistent service delivery across all 14 Meridian Health Partner sites.

2.0 Scope of Service - Conditions Accepted

2.1 The Meridian Health Partners Cardiology department provides outpatient diagnostic, management, and follow-up care for adult patients presenting with:

2.1.1 Hypertension: New diagnoses or management of existing essential hypertension, including those with Systolic Blood Pressure consistently exceeding 140mmHg or Diastolic Blood Pressure consistently exceeding 90mmHg, without acute end-organ damage.

2.1.2 Dyslipidemia: Management of primary and secondary hypercholesterolemia and hypertriglyceridemia.

2.1.3 Coronary Artery Disease (CAD): Stable angina pectoris, post-myocardial infarction follow-up (after initial tertiary care stabilization), and risk factor modification.

2.1.4 Heart Failure: New York Heart Association (NYHA) Functional Class I-III heart failure, stable and optimized on medical therapy.

2.1.5 Arrhythmias: Stable atrial fibrillation/flutter, supraventricular tachycardia (SVT), and premature ventricular/atrial contractions (PVCs/PACs) requiring diagnostic workup or ongoing medical management.

2.1.6 Valvular Heart Disease: Mild to moderate valvular stenoses or regurgitations requiring monitoring, excluding critical or rapidly progressive cases necessitating immediate surgical evaluation.

2.1.7 Peripheral Artery Disease (PAD): Stable claudication or asymptomatic PAD, requiring diagnostic evaluation and medical management.

3.0 Scope of Service - Conditions Not Accepted for Outpatient Management

3.1 The following conditions are explicitly outside the scope of routine outpatient Cardiology services at Meridian Health Partners and require immediate referral to an Emergency Department or a higher level of care:

3.1.1 Acute Coronary Syndromes: Unstable angina, acute myocardial infarction (STEMI or NSTEMI).

3.1.2 Acute Decompensated Heart Failure (ADHF): Patients presenting with acute worsening of heart failure symptoms requiring intravenous diuretics or inotropic support.

3.1.3 Hypertensive Crisis: Systolic Blood Pressure (SBP) consistently exceeding 180mmHg OR Diastolic Blood Pressure (DBP) consistently exceeding 120mmHg, especially with evidence of acute end-organ damage.

3.1.4 Life-Threatening Arrhythmias: Sustained ventricular tachycardia (VT), ventricular fibrillation (VF), or rapidly deteriorating supraventricular tachyarrhythmias causing hemodynamic instability.

3.1.5 Acute Pulmonary Embolism (PE).

3.1.6 Complex Congenital Heart Disease: Requiring specialized pediatric or adult congenital cardiology expertise not available within Meridian Health Partners. Exceptions may be granted for stable adult patients with simple shunts upon direct consultation with Dr. Elara Vance.

4.0 Typical Appointment Duration

4.1 The standard scheduling blocks for Cardiology appointments are as follows:

4.1.1 New Patient Consultations: 45 minutes.

4.1.2 Follow-up Consultations: 20 minutes.

4.1.3 Echocardiogram: 30 minutes.

4.1.4 Treadmill Stress Test: 75 minutes.

4.1.5 Pharmacologic Stress Test: 90 minutes.

5.0 Patient Preparation Instructions

5.1 General Consultation:

5.1.1 Patients must bring a complete and current list of all medications, including dosages and frequency, at every appointment.

5.1.2 Patients should be prepared to discuss their full medical history, including any previous cardiac events, surgeries, and family history.

5.1.3 Relevant medical records, imaging reports, or laboratory results from outside Meridian Health Partners should be brought or forwarded prior to the appointment.

5.2 Cardiac Stress Test (Treadmill or Pharmacologic):

5.2.1 Patients must fast for at least 4 hours prior to the test.

5.2.2 Patients must avoid all caffeine-containing products (e.g., coffee, tea, soda, chocolate, energy drinks, certain medications) for 24 hours prior to the test.

5.2.3 Patients should wear comfortable, loose-fitting clothing and walking shoes.

5.2.4 Specific medications (e.g., beta-blockers, calcium channel blockers, nitrates) may need to be held prior to the test; patients will receive individualized instructions from their referring physician or the Cardiology scheduling coordinator.

5.3 Echocardiogram:

5.3.1 No specific fasting or medication restrictions are required.

5.3.2 Patients should wear loose clothing to allow for easy access to the chest area.

6.0 Meridian Health Plans - Cardiology Coverage

6.1 The following outlines typical coverage for Cardiology services across the four Meridian Health Partner insurance plans:

6.1.1 Meridian Basic Plan: Covers initial consultations, follow-up consultations, electrocardiograms (EKG), and basic blood laboratory tests related to cardiac health (e.g., lipid panel, basic metabolic panel).

6.1.2 Meridian Standard Plan: Covers all services under Meridian Basic, plus standard echocardiograms, 24-hour and 48-hour Holter monitoring, and basic treadmill stress tests.

6.1.3 Meridian Premium Plan: Covers all services under Meridian Standard, plus cardiac computed tomography angiography (CCTA), pharmacologic stress tests, up to 7-day Holter monitoring, and remote monitoring for basic pacemakers.

6.1.4 Meridian Executive Plan: Covers all services under Meridian Premium, plus cardiac magnetic resonance imaging (MRI), advanced electrophysiology consultations, and remote/in-person checks for complex cardiac devices (e.g., ICDs, CRT-Ds) at designated Meridian Health Partner sites.

7.0 Department Referral Turnaround Commitment

7.1 The Cardiology department commits to the following processing times for all incoming referrals:

7.1.1 Non-Urgent Referrals: Acknowledgement of receipt within 24 business hours. Scheduling of the initial consultation within 5 business days of acknowledgement.

7.1.2 Urgent Referrals: (e.g., new onset NYHA Class III heart failure, rapidly progressive symptoms of stable angina): Acknowledgement of receipt within 4 business hours. Scheduling of the initial consultation within 2 business days of acknowledgement.

7.2 All referral acknowledgement and scheduling communications will be managed by the Cardiology Intake Coordinator, Mr. Jordan Chen.